



OFFICIAL

Consumer Safety Planning Procedure

1. Scope

Site	Operational Area	Applicable to
Armadale Kalamunda Group	Mental Health Services	Medical, Nursing, Allied Health

2. Purpose

Armadale Kalamunda Group (AKG) Mental Health Service is continually working towards improving all aspects of its service delivery by adopting consumer centred and recovery focused strategies, in collaboration with consumers, to ensure risks are minimised and managed safely and effectively.

It is recognised that each person is different in the range of events, behaviours or environments which trigger arousal or behaviours and those which they find calming. Staff on the inpatient unit will work with consumers to identify these on admission.

The Consumer Safety Plan (CSP) form is designed to assist the consumer and the treating team to enter into a 'partnership of safety' by identifying stimuli which are likely to lead to increased arousal, and to apply strategies such as avoiding, removing or utilising activities which are calming.

The CSP will assist staff to identify triggers and early warning signs of increasing arousal for each consumer, and to apply the calming strategies which the consumer has nominated as helpful.

This will assist the treating team in:

- Early intervention and individualisation of care.
- Protecting the dignity and freedom of patients.
- Promoting a safe environment for staff and patients
- Boosting therapeutic relationships between patients and staff.

Note: Consultation with other people such as relatives, parents, carers and nominated persons in the development of the CSP is essential and in accordance with the principles of the Mental Health Act 2014.

3. Background

The Chief Psychiatrist of Western Australia: Standards for Clinical Care as required under Section 547 of the Mental Health Act 2014 - Standard: Consumer and Carer Involvement in Individual Care, require mental health services to be responsive to consumer and carer input and needs:

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Mental health services are required to work collaboratively and in partnership with consumers and/or carers irrespective of whether the consumer is a voluntary or involuntary patient.
- There should always be an emphasis on recovery orientated care. This involves active partnership with consumers and carers by services.
- All services must have regard to the principles in the Charter of Mental Health Care Principles.
- It is a requirement that consumers, carers, clinicians and other associated staff operate in an environment of mutual respect.
- All safety planning procedures must be sensitive to diverse sexualities and genders (LGBTQIA+). They must also be culturally sensitive and appropriate for consumers from an Aboriginal or ethnoculturally and linguistically diverse background.

4. Procedure

4.1 Risk assessment and Management Plan

The purpose of assessing risk is to establish and agree on the actions to be undertaken in the clinical care of each mental health consumer and to ensure the safety of consumers, carers, staff and members of the public.

- The RAMP is to be completed using WebPSOLIS.
- To be conducted on:
 - at activation in ED
 - at admission
 - prior to, and on return from, new leave episodes.
 - It may not always be necessary to undertake a new formal risk assessment for every episode of leave once the risks for the type of leave episode have been identified, e.g. the consumer going for a coffee. Clinical discretion and judgement should be used on a case-by-case basis
 - prior to change of setting, transfer of care and/or discharge
 - following a change in clinical status, such as increased acuity
- In the inpatient setting the RAMP is to be reviewed and updated if necessary, no less than weekly, as risks may change over time. Depending on the circumstances it may be appropriate to also update the Consumer Safety Plan accordingly.

4.2 Consumer Safety Plan

- The Consumer Safety Plan will be completed utilising the SMHMR932 SSCD form.
- In the Community the CSP is to be offered to the consumer by the Care Coordinator to complete as part of the mental health assessment requirements within four weeks of admission. If the CSP form is not appropriate or declined by the consumer, a safety plan will be incorporated into the Treatment Support Discharge Plan (TSDP) for consumers who have a suicide risk or may be at risk of becoming aggressive or violent utilising the best practice principles as outlined in supporting information.

- On admission to CRHTT a safety plan will be incorporated into the Treatment Support Discharge Plan (TSDP).
- On admission to the Armadale Mental Health Service inpatient unit, consumers will be asked to complete a Consumer Safety Plan within 72 hours, in collaboration with their admitting nurse.
- If the consumer declines to complete a CSP or their mental state prevents them from providing information, this should be documented in the health record. Completion of the CSP should be explored with the consumer each subsequent 24 hours.
- The admitting nurse will review the CSP with the consumer if completed in the community setting and make appropriate changes if necessary.
- The CSP is placed in the front of the consumer's health record whilst an inpatient of the Armadale Mental Health Service.
- Where the CSP is completed by the consumer in the community or while admitted to CRHTT, the CSP will be attached to the TSDP in the front of the consumer's health record.
- The CSP will be reviewed, and requirements for update identified, as part of the inpatient weekly ward round, community clinical team review meeting, or CRHTT weekly MDT meeting.
 - Where no changes to the CSP are required, the review should be recorded in the consumer's Health Record.
 - Where changes are required, this will be reflected in the CSP and the Health Record.
- A review of the CSP and its efficacy should be reviewed with the consumer after any incident and/or change in mental state.
 - A seclusion and/or code black event will trigger a review and update of the Consumer Safety Plan.
- A review of the CSP is to be conducted by the Social Worker upon the completion of a Social Work Assessment.

5. Policy context

The following documents are required to give effect to this procedure:

- Legislative
 - Mental Health Act 2014
- Chief Psychiatrist of Western Australia
 - [Standards for Clinical Care](#)
- HDWA
 - [Safety Planning for Mental Health Consumers Policy](#)
 - ['Triage to Discharge' Mental Health Framework for Statewide Standardised Clinical Documentation](#)
- EMHS
 - [Rights of Patients in Mental Health EMHS: 170](#)

6. Supporting information

The following documents support the implementation of this procedure:

- HDWA

- [Safety Planning Procedures for Mental Health Consumers Procedure](#)
- EMHS
 - [Prevention and Management of Workplace Aggression and Violence](#)
- AKG
 - [Safe and Supportive Observations](#)
 - [Assessment Requirements for Mental Health Consumers Procedure](#)
 - [Mental Health Restraint Policy](#)
 - [Mental Health Seclusion Policy](#)

7. Compliance, monitoring and evaluation

Compliance against this policy will be evaluated results from the Armadale Mental Health Service Documentation Audit.

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.30 Safe environment.
- Standard 5 – Action 5.34. The health service organisation has processes to support collaboration with patients, carers and families
- Standard 5 – Action 5.36. Minimising restrictive practices: seclusion

9. Policy owner (relating to these procedures)

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

Title: Coordinator of Nursing – Mental Health
 Department: Mental Health
 Email: Dene.Olive@health.wa.gov.au

10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V5	29 01 2026	29 01 2029	Full review, minor changes only.

11. Authorisation

This procedure has been authorised and issued by:

Authorised by	Dene Olive – A/ Service Director Mental Health.
Authorisation date	29 01 2026
Published date	29 01 2026